



MoE - Exit Exam

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Time left 2:58:26

Question 12

Not yet answered

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[Flag question](#)

A 23-year-old male patient who lives on the road side was admitted to a medical ward for acute febrile illness. He developed diaphoresis and palpitation hours after drug administration. On examination, his BP dropped to 80/40mmHg. What is the most appropriate next step in the management of this patient?

- ☐ a. Give IV fluid and dexamethasone
- ☐ b. Reassure the patient and continue the medication
- ☐ c. Send blood culture
- ☐ d. Transfuse with cross-matched blood

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Time left 2:58:12

Question 14

Not yet answered

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[Flag question](#)

A 24-year-old man came to a hospital with complaints of cough, fever and pleuritic chest pain of three days duration. He was treated with unspecified antibiotics for URTI two weeks back. On physical examination, his vital signs were unremarkable, but chest examination showed left middle lung field bronchial breath sound. What is the best initial management option for this patient?

- ☐ a. PO Augumentin plus Azithromycin
- ☐ b. PO Clarithromycin plus Doxycycline
- ☐ c. PO Doxycycline plus Ciprofloxacin
- ☐ d. PO Azithromycin

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Question 16

Not yet answered

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A 25-year-old female patient came to a hospital with fatigue, shortness of breath and bilateral leg swelling of one month duration. She has no previously known chronic illness. Her blood pressure is 90/60 mmHg, pulse rate is 112/min, respiration rate is 18/min and temperature is 37.2 degree centigrade. She has right side pleural effusion, raised JVP, grade 2-3/6 holosystolic murmur at apex and bilateral leg swelling. Echocardiography showed left ventricular cavity diameter of 65 mm (normal value less than 54 mm) and left atrial diameter of 45 mm (normal value less than 38 mm) with ejection fraction of 30percent. What is the most likely underlying disease of the patient?

- ☐ a. Hypertrophic cardiomyopathy
- ☐ b. Myocarditis
- ☐ c. Dilated cardiomyopathy
- ☐ d. Restrictive cardiomyopathy

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Time left 2:58:07

Question 15

Not yet answered

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A 24-year-old man has been on RHZE for the last three weeks for the diagnosis of pulmonary tuberculosis. In the last two week, the patient started to complain of burning and tingling sensation in his feet. What is the most appropriate step in the management of the patient's case?

- ☐ a. Reassurance and continue anti-TB
- ☐ b. Start on pyridoxine and continue anti-TB
- ☐ c. Discontinue only Isonazid and start on pyridoxine plus RZE
- ☐ d. Start on pyridoxine and stop all anti-TB

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Time left 2:57:41

Question 18

Not yet answered

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[Flag question](#)

A 25-year-old male patient came to a neurology clinic for his regular epilepsy follow-up. He has been taking the maximum dose of valproic acid, but he had six episodes of seizure in the last two months. He claimed that he is adherent to his medication. What is the most appropriate next step in the management of this patient?

- ☐ a. Taper Valproic acid and start regular doses of diazepam
- ☐ b. Add other anti-epileptic drug with different mechanism of action
- ☐ c. Add two drugs with similar mechanisms of action
- ☐ d. Discontinue Valproic acid and start other anti-epileptic drug

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Time left 2:57:19

Question 21

Not yet answered

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Flag question

A 26-year-old male patient came to an OPD with complaint of pleuritic chest pain, shortness of breath, low grade fever and cough of two weeks duration. His blood pressure was 110/60mmHg, pulse rate was 86/min, respirations were 22/min and temperature was 37.9 degree centigrade. His Physical examination revealed presence of left lower chest dullness and absent air entry. Laboratory findings show WBC count of 12000/mm³, ESR of 55mm/hr and pleural fluid analysis shows WBC count of 1000 /mm³ with Lymphocyte of 70percent. What is the mostly likely diagnosis for this patient?

- ☐ a. Tuberculosis pleurisy
- ☐ b. Para pneumonic effusion
- ☐ c. Hepatic hydrothorax
- ☐ d. Malignant effusion

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Question 20

Not yet answered

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A 25-year-old young man came to an emergency OPD after having dry cough, running nose fever and headache of three days duration. He did not have history of chronic lung problems and he is not smoker. His vital signs were within the normal range except temperature of 38.4 degree centigrade, he has clear chest and normal mental status. What is the most appropriate next step in the management of this patient?

- ☐ a. Do not give any medications
- ☐ b. Prescribe antibiotics
- ☐ c. Prescribe antiviral
- ☐ d. Prescribe antipyretics

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Time left 2:56:54

Question 24

Not yet answered

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Flag question

A 27-year-old patient came to an emergency unit complaining about nausea, vomiting, thirst, abdominal pain and shortness of breath for the last two days. On evaluation, temperature was 36.7 degree centigrade, respirations were 28/min, pulse rate was 116/min and blood pressure was 80/60mmHg. He is lethargic; however, he has no remarkable finding in other systems. His laboratory results reveal; WBC count of 4.8×10^3 /microlitre, platelet count of 156×10^3 /microlitre, hemoglobin level of 11.4g/dl, liver and kidney functions were normal, serum K^+ 4.5mmol/L (NR; 3.5-5.5mmol/L), serum Na^+ 140mmol/L (NR; 136-146mmol/L), RBS 560mg/dl and urine analysis shows ketone +3. What is the most appropriate initial step in the management of this patient?

- ☐ a. Metformin 1gm
- ☐ b. Start feeding by nasogastric tube
- ☐ c. 0.9percent saline 10 ml/kg/hr
- ☐ d. Ceftazidime 2gm

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Question 23

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A 27-year-old male patient came to a medical OPD with a complaint of retrosternal chest pain of two days duration. On evaluation, his temperature is 36.5 degree centigrade, respiratory rate is 25/min, pulse rate is 92/min, and blood pressure is 100/60mmHg. There are no remarkable findings in other systems. Laboratory results show hemoglobin level of 12g/dl, WBC count of 5×10^3 /microlitre, and platelet count of 250,000/microlitre. Cardiac biomarkers are in the normal range. Chest X-ray findings are normal. ECG shows diffuse ST-segment elevations with upward concavity in leads I, II, AVF, and v2 to v6. What is the most likely diagnosis of this patient?

- ☐ a. Acute coronary syndrome
- ☐ b. Pericarditis
- ☐ c. Pulmonary thromboembolism
- ☐ d. Pneumonia

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Question 27

Not yet answered

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A 28-year-old male patient came to a health center after noticing skin discoloration on his back for unknown duration. Five hypo-pigmented skin lesions with loss of sensation on the lesion and enlarged ulnar nerves were found during examination. What is the most likely diagnosis?

- ☐ a. Leprosy
- ☐ b. Ulnar nerve neuropathy
- ☐ c. Tinea corporis
- ☐ d. Pityriasis alba

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Question 30

Not yet answered

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A 30-year-old female patient comes to a medical referral clinic complaining of bilateral recurrent pain and swelling of her hand joints for two years. She has no skin rash or photosensitivity. Her vital signs are in the normal range. Musculoskeletal examinations reveal swan neck like deformity of her hands. What is the most likely diagnosis of the patient?

- ☐ a. Osteoarthritis
- ☐ b. Reactive arthritis
- ☐ c. Gout
- ☐ d. Rheumatoid arthritis

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Question 35

Not yet answered

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A 32-year-old male patient came to a hospital with the complaints of cough and severe left side chest pain while breathing for the last one week. His blood pressure is 110/60 mmHg, pulse rate is 100/min, respirations are 32/min and temperature is 38.4 degree centigrade. Respiratory system examination showed dullness and absent air entry over the left lower half of the lung fields. What is the most appropriate initial investigation for the patient?

- ☐ a. Echocardiography
- ☐ b. Chest x-ray
- ☐ c. Thoracentesis
- ☐ d. Complete blood count

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Question 37

Not yet answered

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A 33-year-old woman came to a clinic with complaints of decreased urine output and foamy urine of one week duration. On physical examination, a blood pressure of 150/90 mmHg, peri orbital edema and malar rash were noted. Investigation results show a urine analysis blood of +2, protein of +3 and many RBC cast, 24-hour urine protein of 2.5g and serum creatinine of 1.3mg/dl. What is the most likely cause of the patient's condition?

- ☐ a. Lupus nephritis
- ☐ b. Post streptococcus glomerulonephritis
- ☐ c. IgA nephropathy
- ☐ d. Focal segmental glomerulosclerosis

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Question 41

Not yet answered

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Flag question

A 35-year-old female patient came to a hospital with anterior neck swelling of five years duration. Since the last three month, she has started to experience palpitation and has a history of hot intolerance with no history of bone pain, cough or yellowish discoloration of the eye. On examination, her pulse rate is 110/min, blood pressure is 120/70mmHg, and there is lid lag and lid retraction. On thyroid examination, there is diffusely enlarged thyroid measuring of 4*3cm;the lower border is reachable. She is investigated with thyroid function test and the results show that TSH is 0.3micro unit/mL(Normal; 0.5–5 micro unit/mL), Total T4 is 180 nmol/L(Normal; 55–150 nmol/L), Total T3 is 5 nmol/L(Normal; 1.5–3.5 nmol/L). What is the most likely diagnosis of the patient?

- ☐ a. Toxic multinodular goiter
- ☐ b. Toxic Adenoma
- ☐ c. Graves' disease
- ☐ d. Follicular neoplasm

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Question 45

Not yet answered

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A 35-year-old woman presented with complaints of foul smelling greasy diarrhea, fatigability and weight loss. On examination, she is emaciated and has pale conjunctiva. Laboratory is notable for hemoglobin level of 8.5g/dl with MCV of 105fL. What is the most likely nutrient deficiency the patient has?

- ☐ a. Iron
- ☐ b. Cobalamin
- ☐ c. Copper
- ☐ d. Phosphorus

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Question 44

Not yet answered

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 [Flag question](#)

A 35-year-old male patient came to an emergency OPD complaining of high grade fever, headache and neck pain of three days duration. He has been admitted to a private hospital for three days taking unidentified medications but had no improvement. His temperature is 38.2 degree centigrade, pulse rate is 100/min and respirations are 24/min. On neurologic evaluation, he has neck stiffness and kerning sign is positive. Laboratory studies show; WBC count of $16.3 \times 10^3/\text{mm}^3$, platelet count of $480 \times 10^3/\text{mm}^3$, hemoglobin level of 14g/dl, there are 450 cells in CSF of which 75percent are neutrophils and he is positive for HIV. What is the most appropriate empiric management for the patient?

- ☐ a. Ceftazidime and Metronidazole
- ☐ b. Ceftriaxone, Prednisolone and Chloramphenicol
- ☐ c. Ceftriaxone, Vancomycin and Ampicillin
- ☐ d. Cefipime and Dexamethasone

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Question 51

Not yet answered

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A 38-year-old daily laborer from Humera presents with complaints of fatigue, abdominal swelling and early satiety for the past six months. On physical examination, he is wasted, has pale conjunctiva and splenomegaly. His hemoglobin level is 9 g/dl, WBC count is 3000/mm³ and platelet count is 100000/mm³. His splenic aspirate microscopy revealed amastigote. PIHTC non-reactive (NR; Hemoglobin 13.3-16.2g/dl, WBC count 3.54–9.06 × 10³/mm³, platelet count 165–415 × 10³/mm³). What is the first line drug for the management of this patient?

- ☐ a. amphotercin B + paramomycin
- ☐ b. miltefosine+amphotercin B
- ☐ c. amphotercin B + sodiumstibulogluconate
- ☐ d. sodium stibulgluconate +paramomycin

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Question 53

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A 40-year-old female patient came to an OPD with the complaint of amenorrhea of three months duration. She also had frequent abortion for which she visited many private clinics and did not get cure. She also complained hot intolerance and significant weight loss. Her pulse rate is 120/min. On examination, she has lid lag and exophthalmia, diffused anterior neck swelling which moves with respiration, and thickened skin over the dorsum of the foot and pre-tibial region. What is the most likely explanation for these findings?

- ☐ a. Hashimoto's thyroiditis
- ☐ b. Secondary thyrotoxicosis
- ☐ c. Subacute thyroiditis
- ☐ d. Graves disease

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Question 55

Not yet answered

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A 41-year-old man came to a hospital with a three weeks history of generalized body swelling that started from the legs and progressed to the whole body. He also has shortness of breath at rest and orthopnea of three pillows equivalent. On examination, his blood pressure is 130/80 mmHg, pulse rate is 120/min, respirations are 32/min, temperature is 35.8 degree centigrade, he has bilateral basal fine crepitation and S3 gallop, and TVLS is 14cm. Investigation showed Creatinine level of 2.1 mg/dl. What is the best initial treatment for the patient?

- ☐ a. Enalapril
- ☐ b. Metoprolol
- ☐ c. Hydrochlothizide
- ☐ d. Furosemide

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Question 65

Not yet answered

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 [Flag question](#)

A 55-year-old man came to an OPD with progressive left side chest pain. The pain starts while performing activities and improves during resting. He has smoked about 20 pack years. Physical examination showed normal system findings, but his blood pressure is 140/90 mmHg. What is the most likely cause of the patient's chest pain?

- ☐ a. Subacute pericarditis
- ☐ b. Atherosclerotic coronary artery disease
- ☐ c. Chronic pulmonary thromboembolism
- ☐ d. Obstructive valve disease

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Question 68

Not yet answered

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 Flag question

A 60-year-old female known diabetic patient on follow-up was found to have hypertension. She is not symptomatic, but her blood pressure was in the hypertensive range for several months. She is taking ant-diabetic medications properly and her glycemic control is good. Currently, her blood pressure is 160/100 mm Hg. Laboratory investigation showed protein +2 in the urine otherwise normal. What is the preferred drug to start for blood pressure control?

- ☐ a. Amlodipine
- ☐ b. Hydrochlorothiazide
- ☐ c. Atenolol
- ☐ d. Enalapril

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Question 66

Not yet answered

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 [Flag question](#)

A 55-year-old man came to an outpatient clinic with a complaint of intermittent epigastric burning pain which is worsened by taking food and awakes him from sleep for the last two months. He has no weight loss and any history of drug intake. Physical examination shows mild epigastric tenderness. Laboratory studies show hemoglobin level of 10.5g/dl, WBC count of 7500/mm³ and positive stool helicobacter antigen. Upper GI endoscopy shows duodenal ulcer. What is the best management for the patient?

- ☐ a. Start on H2 blocker
- ☐ b. Start on PPI +Clarithromycin +Metronidazole
- ☐ c. Start on proton pump inhibitor
- ☐ d. Start on H2 blocker +Clarithromycin +Metronidazole

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Question 52

Not yet answered

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 [Flag question](#)

A 38-year-old female patient, who is on treatment for Crohn's disease for two years, came to a medical referral clinic complaining about weakness, palpitation, forgetfulness and headache of two months duration. On evaluation, she has tachycardia and absent deep tendon reflexes and position and vibration sensation. Her blood cell count shows WBC of $3.8 \times 10^3/\text{microlitre}$, platelet of $350 \times 10^3/\text{microlitre}$, and hemoglobin of 8.0g/dL. On examination of peripheral blood smear, there are three neutrophils with seven lobes. What is the most appropriate initial management for this patient?

- ☐ a. Prednisolone
- ☐ b. Cyanocobalamine
- ☐ c. Iron sulfate
- ☐ d. Immunotherapy

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Question 57

Not yet answered

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A 48-year-old female patient was admitted to a medical ICU with the diagnosis of NYHA Class-IV Stage-C CHF secondary to chronic rheumatic valvular heart disease. The physicians and nurses in the ICU were unable to secure IV line. Where is the most appropriate site for IV access in the patient?

- ☐ a. Saphenous vein cut down
- ☐ b. Intraosseous route
- ☐ c. Scalp vein
- ☐ d. Jugular vein cut down

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Question 60

Not yet answered

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A 52-year-old smoker was admitted and treated for heart failure secondary to ischemic heart disease. He improved significantly with no complaints. The patient's current vital signs are within the normal range. What is the best advice on discharge for long term benefit?

- ☐ a. Perform vigorous exercises.
- ☐ b. He can continue smoking as far as he continues his medication.
- ☐ c. He can start low dose alcohol for preventive effect.
- ☐ d. Stop smoking and continue his medication.

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Question 58

Not yet answered

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 [Flag question](#)

A 49-year-old man came with one day history of severe shortness of breath, cough and confusion. He is a known asthmatic patient with irregular follow-up and not adherent to medications. On examination, his blood pressure is 140/90 mmHg, respirations are 40/min and SPO₂ is 70percent at room air. The patient was admitted with emergency treatment protocol. What is the best initial treatment for this patient?

- ☐ a. Long acting beta agonists
- ☐ b. Systemic steroids
- ☐ c. Short acting beta agonists
- ☐ d. Magnesium sulphate

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Question 59

Not yet answered

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A 50-year-old female patient presents with a complaint of cough of two years duration. The cough is productive of small tenacious whitish sputum that is present in most days of the week. She also complains of occasional chest tightness, exertional dyspnea, fatigue and loss of appetite of same duration. She has no history of cigarette smoking. She uses wood for cooking. Physical examination shows scattered wheezing over chest fields. Chest x-ray shows hyper inflated lungs with flattening of diaphragm. What is the most likely diagnosis of this patient?

- ☐ a. Bronchiectasis
- ☐ b. COPD
- ☐ c. Obliterative bronchiolitis
- ☐ d. Bronchial asthma

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Question 61

Not yet answered

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A 54-year-old man came to a hospital with a complaint of vomiting of coffee ground matter of five hours duration. He has long standing epigastric pain, easy fatigability and dark tarry stool. At presentation, his blood pressure is 85/60mmHg and pulse rate is 120/min. His abdominal examination did not reveal remarkable finding. His laboratory findings show WBC count of $4.3 \times 10^3/\text{mm}^3$, platelet count of 178×10^3 and hemoglobin level of 7g/dl. What is the most appropriate initial management for the patient?

- ☐ a. Resuscitate and prepare for emergency laparotomy
- ☐ b. Secure IV line and send for upper GI endoscopy immediately
- ☐ c. Resuscitate with double IV line and prepare cross matched blood
- ☐ d. Put him on maintenance fluid and do NG tube lavage

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Question 62

Not yet answered

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A 55-year-old diabetic patient was brought to a medical emergency OPD after sudden loss of consciousness while he was on bed two days ago. On evaluation; his temperature was 36.0 degree centigrade, pulse rate was 77/min, respirations were 23/min, and blood pressure was 160/100mmHg. On neurologic examination, the power on the left upper and lower extremity was 0/5 and GCS was 15/15. What is the most appropriate next step in the patient's care?

- ☐ a. Do brain CT
- ☐ b. Do brain MRI
- ☐ c. Start Atrovastatin
- ☐ d. Start Asprin

Doctor of Medicine

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Question 63

Not yet answered

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A 55-year-old female patient came to an emergency OPD with complaints of shortness of breath and right side chest pain of two days duration. Her temperature is 36.8 degree centigrade, respirations are 28/min, pulse rate is 104/min, and blood pressure is 80/50 mmHg. Cardio-respiratory examination shows that the lower posterior chest is dull to percussion and JVP is raised. CT angiography shows pulmonary artery thrombosis. What is the most effective management of this patient?

- ☐ a. IV fibrinolytic therapy
- ☐ b. Systemic glucocorticoids
- ☐ c. High dose Furosemide
- ☐ d. Oral anti-platelet

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Question 70

Not yet answered

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 [Flag question](#)

A 62-year-old man, who is a known HIV patient on AZT/3TC/EFV for the last two years with a good drug adherence, came to an adult OPD with complaints of headache and recurrent seizure of three weeks duration. Previously he was asymptomatic. His CD4 count was 400/microlitre six months back and currently it is 100/microlitre. What is the most likely explanation for his current problem?

- ☐ a. Acute stroke
- ☐ b. Treatment failure
- ☐ c. Drug side effects
- ☐ d. IRIS

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Doctor of Medicine

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Question 72

Not yet answered

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A 65-year-old, a known DM patient on metformin for last six years, presented with complaint of facial swelling for the last six weeks. He was diagnosed with Diabetic retinopathy before. Vital signs are in the normal range except his blood pressure is 150/95mmHg. He had periorbital and leg edema. On investigation, urinalysis albumin +4 blood +1, Cr-2.5mg/dl, 24 hour urine protein of 3.2g and HgA1c 8percent. What is the most appropriate management of this patient?

- ☐ a. Start on Furosemide
- ☐ b. Start on steroid
- ☐ c. Start on antibiotic
- ☐ d. Start on Enalapril

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Doctor of Medicine

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Question 82

Not yet answered

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A nurse working at a hospital sustained needle stick injury while carrying order for the patient with chronic liver disease with positive for hepatitis B surface antigen. The nurse had no history of liver disease and serology for hepatitis B is negative. What is the best management for this scenario?

- ☐ a. Do anti-HBs level and treat accordingly
- ☐ b. Reassurance and no treatment
- ☐ c. Give single dose of HBIG and single dose of HBV vaccine
- ☐ d. Give single dose of HBIG and full course of HBV vaccine

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Question 82

Not yet answered

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A nurse working at a hospital sustained needle stick injury while carrying order for the patient with chronic liver disease with positive for hepatitis B surface antigen. The nurse had no history of liver disease and serology for hepatitis B is negative. What is the best management for this scenario?

- ☐ a. Do anti-HBs level and treat accordingly
- ☐ b. Reassurance and no treatment
- ☐ c. Give single dose of HBIG and single dose of HBV vaccine
- ☐ d. Give single dose of HBIG and full course of HBV vaccine

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Question 77

Not yet answered

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A five-year-old male child is brought to a hospital with complaints of failure to use his left hip joint and pain of 10 days duration. Physical examination elicited a temperature of 38.9 degree centigrade, pulse rate of 112/min and slightly swollen right hip joint. He had also tender and restricted passive and active movements. The X-ray and ultrasound findings are normal. What is the most appropriate initial step in the management of this child?

- ☐ a. IV antibiotics and open joint biopsy
- ☐ b. IV antibiotics and follow change with ESR
- ☐ c. IV antibiotics and joint fluid gram stain and culture
- ☐ d. IV antibiotics and do CRP

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Question 79

Not yet answered

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 [Flag question](#)

A known diabetic patient for the last 15 years came to a regular OPD with the complaint of left calf pain of two months duration. The pain was aggravated during walking and relieved during rest. What is the most likely diagnosis of the patient?

- ☐ a. Diabetic neuropathy
- ☐ b. Deep vein thrombosis
- ☐ c. Varicose vein
- ☐ d. Peripheral arterial disease

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Question 84

Not yet answered

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A physician was evaluating a CLD patient who is on propranolol and has had endoscopic band ligation one month back for primary prophylaxis of variceal bleeding. His blood pressure is 100/60mmHg and pulse rate is 92/min. On laboratory investigation, Hemoglobin level is 12.5g/dl and platelet count is 135,000/mm³. What clinical parameter is needed to optimize the propranolol dose?

- ☐ a. Platelet count
- ☐ b. Hemoglobin
- ☐ c. Pulse rate
- ☐ d. Blood pressure

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